DEMOGRAPHICS

Gary Planchon

DOB: 19 Sep 1938
Marital Status:
Sex: Male
Languages:

MRN:
Maiden Last:
SSN:
Driver's License:
Race:
Ethnicity:

Preferred Communication: Unknown

Home Address: 4965 HILDAGO WAY
LAS VEGAS, NV 89121

Home Phone: (702) 451 - 7343
Mobile Phone:
Personal Email:
Other Email:

Disclosure Notes:

Additional Info

Mother's Maiden Last Name:
Birth Order:
Citizenship(s):
Education:

Birth Place:
Nationality:
Religious Affiliation:

Date of Death:

Cause of Death:

Pharmacies: Walmart Pharmacy 3350: 5198 Boulder Highway, LAS VEGAS (BOULDER) 89122 (702) 434 - 1915 (o) (702) 434 - 1958 (f) Primary

Primary Provider:

Referred By:

Notes:

Contacts

Responsible Party: Self
Relationship to Patient: Self
Name: GARY PLANCHON
Home Address: 4965 HILDAGO WAY
LAS VEGAS, NV 89121

Home Phone: (702) 451 - 7343
Work Phone:
Mobile Phone:
Other Phone:
Personal Email:
Work Email:
Other Email:
Notes:

Emergency Contact: Responsible Party
Relationship to Patient: Self
Name: GARY PLANCHON
Home Address: 4965 HILDAGO WAY
LAS VEGAS, NV 89121

Home Phone: (702) 451 - 7343
Work Phone:
Mobile Phone:
Other Phone:
Personal Email:
Work Email:
Other Email:
Notes:

Insurance

Company Name:	Medicare of Nevada	Insured Party:	Self
Plan Name:		Name:	GARY PLANCHON
Address:	Attn Novitas Solutio P.O. Box 3385 Mechanicsburg PA 17055	Relationship to Patient:	Self
		Home Address:	4965 HILDAGO WAY LAS VEGAS, NV 89121
Phone Number:	8006334227	Home Phone:	(702) 451 - 7343
Fax Number:		Work Phone:	
Contact Person:		Mobile Phone:	
Plan Type:		Other Phone:	
Group Name:		Personal Email:	
Policy Number:	9AN2K77MT12	Work Email:	
Group Number:		Other Email:	
Start/End Date:	18-Mar-2025 – 18-Mar-2025		
Copay:	$0.00		

Gary Planchon
MRN :

Birthday : **1938-09-19**
Visited on: 2026 Apr 06 10:00 (Age at visit: 87 years)

Phone : **7024517343**
Electronically signed by: JEFFERY OLSEN, DO on 2026-04-07 08:51 AM

CC

APLUS

Follow up visit

Initiation of Visit: This visit was initiated by ¬¬¬¬the patient_. The chief complaint is needing an in-home healthcare provider_.
MEDICAL NECESSITY: This visit with a FTF encounter with the patient in the home is not a visit of convenience. Patient is seen in the home because there is a medical necessity for this visit to be conducted in the patient's environment. It is medically necessary and safer for the patient to be seen in the patient's own environment today.

Chief Complaint
Follow-up for pneumonia, decreased appetite and weight loss, lower back pain from hitting something in bedroom

Subjective

History of Present Illness
The patient returns for follow-up of pneumonia, reporting significant improvement in symptoms. His nasal congestion has cleared up quite a bit since taking his prescribed medication, and he is no longer coughing much. However, he has experienced decreased appetite and weight loss since completing his antibiotic course one week ago. He describes eating very little throughout the day - typically just a bowl of cereal with minimal milk and coffee in the morning, minimal snacking during the day, and small portions at dinner. He has been nodding off frequently and appears to have lost noticeable weight, though the exact amount is unknown.

The patient also reports a new complaint of lower back tenderness after hitting something in his bedroom. The area has some swelling and feels tender to him, though it does not cause significant pain when pressed. He can feel pressure when the area is touched but describes no radiating pain.

Additionally, he experienced diarrhea while on his medication, but this has since resolved and his bowel movements have returned to normal.

Medications and Supplements
- Medicine for pneumonia
- Recently completed course. Caused diarrhea which has resolved.

Review of Systems
General: Positive for decreased appetite and weight loss.
HEENT: Positive for nasal congestion (improved).
Respiratory: Positive for cough (resolved).
Gastrointestinal: Positive for diarrhea (resolved).
Musculoskeletal: Positive for lower back tenderness.

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Medications

mirtazapine 7.5 mg tablet, 1 tab by mouth daily
doxycycline hyclate 100 mg tablet, 1 tab orally q12h x 7 days
azithromycin 250 mg tablet, 2tabs (500mg) PO on day 1, then 1tab (250mg) once daily x 4 days
albuterol sulfate 1.25 mg/3 mL solution for nebulization, 3 mL inhaled every 4 to 6 hours prn shortness of breath or wheezing
albuterol sulfate 1.25 mg/3 mL solution for nebulization, 3 mL inhaled every 20 minutes prn shortness of breath or wheezing; for 3 doses
albuterol sulfate 1.25 mg/3 mL solution for nebulization, 3 mL inhaled every 20 minutes prn shortness of breath or wheezing; for 3 doses
albuterol sulfate 1.25 mg/3 mL solution for nebulization, 3 mL inhaled every 20 minutes prn shortness of breath or wheezing; for 3 doses
dextromethorphan-guaifenesin 10 mg-100 mg/5 mL oral liquid, TAKE 10MLS PO EVERY 6HRS AS NEEDED FOR COUGH
Calcium 500 + D 500 mg-10 mcg (400 unit) tablet, 1 tab(s) orally once a day
hydroCHLOROthiazide 25 mg tablet, 1 tab(s) orally once a day
montelukast 10 mg tablet, 1 tab(s) orally once a day at bedtime
metoprolol tartrate 25 mg tablet, 1 tab(s) orally once a day
hydrOXYzine HCL 25 mg tablet, 1 tab(s) orally once a day at bedtime
metFORMIN 500 mg tablet, 1 tab(s) orally once a day
zolpidem 10 mg tablet, 1 tab(s) orally every evening at bedtime

Allergies

Augmentin ( )

Mental/Functional

The patient's speech was normal, sharing conversation with normal laryngeal efforts. Appropriate mood and affect were seen on exam. Thought processes were logical, relevant, and thoughts were completed normally. Thought content was normal. Thought content was normal with no psychotic or suicidal thoughts. The patient's judgement was realistic with normal insight into their present condition. Mental status included: correct time, place, person orientation, normal recent and remote memory, normal attention span and concentration ability. Language skills included the ability to correctly name objects. Fund of knowledge included normal awareness of current and past events.

Vitals

Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2
			136/62		**61**	**19**	**95.0**

Recorded: 2026 Apr 06 10:00 by: JEFFERY OLSEN -

Objective

Physical Examination
General: Patient appears to have lost weight.
Musculoskeletal: Small lump with mild swelling noted in lower back area where patient reports hitting something. No pain elicited on palpation of the area.

Laboratory, Imaging, and Diagnostic Test Results
- X-ray: Improved compared to previous study, no fluid present, pneumonia cleared

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Assessment

Decrease in appetite (R63.0) Anorexia modified 7 Apr, 2026

Obstruction of bronchus (J44.1) Chronic obstructive pulmonary disease w (acute) exacerbation modified 15 Dec, 2025

Degeneration of retina (H35.9) Unspecified retinal disorder modified 15 Dec, 2025

Cough at rest (R05.9) Cough, unspecified modified 17 Mar, 2025

Activity alteration (Z73.6) Limitation of activities due to disability modified 17 Mar, 2025

Advance directive discussed with patient (Z71.89) Other specified counseling modified 17 Mar, 2025

Keeps losing balance (R26.81) Unsteadiness on feet modified 19 Jan, 2025

Melorheostosis of right lower leg (M85.861) Oth disrd of bone density and structure, right lower leg modified 3 Nov, 2024

Blindness left eye category 3, normal vision right eye (H54.42A3) Blindness left eye category 3, normal vision right eye started 24 Aug, 2024 modified 27 Aug, 2024

Low back pain (M54.50) Low back pain, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

Sedative dependence (F13.20) Sedative, hypnotic or anxiolytic dependence, uncomplicated modified 27 Aug, 2024

Generalized obesity (E66.9) Obesity, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

Hearing loss (H91.90) Unspecified hearing loss, unspecified ear started 24 Aug, 2024 modified 27 Aug, 2024

Patient on oxygen (Z99.81) Dependence on supplemental oxygen started 24 Aug, 2024 modified 27 Aug, 2024

Chronic obstructive pulmonary disease (J44.9) Chronic obstructive pulmonary disease, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

Insomnia (G47.00) Insomnia, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

Anxiety (F41.9) Anxiety disorder, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

Falls (R29.6) Repeated falls started 24 Aug, 2024 modified 27 Aug, 2024

Hyperlipidemia (E78.5) Hyperlipidemia, unspecified started 24 Aug, 2024 modified 27 Aug, 2024

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Benign hypertension (I10) Essential (primary) hypertension modified 27 Aug, 2024

Plan

Patient with resolved pneumonia presenting for follow-up with improved respiratory symptoms but experiencing decreased appetite and weight loss following recent antibiotic treatment.

Pneumonia, resolved
Assessment: X-ray shows improvement with resolution of pneumonia. Patient reports significant improvement in nasal congestion and minimal coughing. No fluid present on imaging. Respiratory symptoms have largely resolved.
Plan:
- Pneumonia has cleared, no further respiratory treatment needed

Decreased appetite and weight loss
Assessment: Patient experiencing decreased appetite and noticeable weight loss following completion of antibiotic treatment one week ago. Currently eating minimal amounts - bowl of cereal with little milk for breakfast, coffee, and small dinner portions with minimal snacking throughout the day. Patient appears to have lost weight on examination. Symptoms likely related to recent antibiotic course and should improve as medication clears from system.
Plan:
- Add Ensure chocolate protein shakes between meals
- Prescribe appetite stimulant (small pill, temporary use)
- Encourage gradual increase in Ensure consumption until finishing full servings
- Monitor weight and appetite improvement

Lower back contusion
Assessment: Patient reports tenderness in lower back after hitting something in bedroom. Physical examination reveals small lump with mild swelling, consistent with contusion from blunt trauma. No significant pain on palpation.
Plan:
- Conservative management for minor contusion
- Monitor for resolution of swelling and tenderness

Advance Care Planning (ACP): A voluntary, face-to-face discussion was held with the patient regarding goals of medical treatment, advance directives, and preferences for care. The discussion lasted 30 minutes.

Topics included: treatment goals, values and preferences, resuscitation status, hospitalization and life-sustaining treatment options, and the role of advance directives (living will, healthcare proxy, power of attorney). Patient (and family, if applicable) were

given the opportunity to ask questions, expressed understanding, and confirmed that care preferences should guide provider recommendations.

Patient was comfortable with the provider's recommendations, which were guided by patient's stated values. Patient consented to this ACP discussion. Patient was informed that documentation may result in a living will, assignment of healthcare proxy, or other advance directive, but completion of these is optional.

CCM- Comprehensive personalized chronic care plan created, revised, and updated, as needed. Provided a comprehensive care plan to set and track progress towards health goals, and support between regular face-to-face visits. Spent 30min with patient.

- Continue Home Health Services to prevent injuries to self, progression of diseases, and inpatient hospitalizations.
• SN to monitor, assess, and for progression of disease.
• SN to evaluate knowledge deficits of Pt and family regarding disease processes, medications, medication regimen, comorbidities, and care plan and educate appropriately.
• SN to take VS and log into book for data and history. Report for BP >160 and >90, HR >120 and >50, RR >35 and >10, T >101.5 and >97.0, BS>350 and >80.
• SN needed d/t high potential for disease progression and inpatient hospitalizations.
• SN to evaluate patient's knowledge deficit and teach accordingly the disease process and management of unstable medical conditions including comorbid factors.
• Report s/s of infection or abnormal wound conditions.
• Pt needs assistance with medication management, observation and assessment for changes in condition.
• Pt will need skilled nursing due to high potential for hospital admission.
• Pt will need skilled nursing for teachings of disease process, diet and new medication regimen.
• PT/OT to evaluate and treat appropriately to increase functionality, decreases debility and morbidity. Increase strength, balance, and cardio/pulmonary function. Improve ability to perform ADL's safely and independently. Patient is to improve and/or return to baseline.

Medications:
- Start taking appetite stimulant medication as prescribed to help improve appetite

Nutrition:
- Drink Ensure protein shakes, preferably chocolate flavor
- You don't have to finish the whole Ensure at once - drink it little by little throughout the day
- Try to eat more during the day to prevent further weight loss

Follow-up:
- Monitor appetite and weight over the next week
- Contact office if appetite does not improve for possible additional medication

Ares Advance Practice
5660 W Flamingo Rd Ste B, Las Vegas, NV 89103-2312

SOAP Note

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Breathing Treatments:
- Start breathing treatments twice daily
- Take one treatment in the morning when you wake up
- Take one treatment around 5:00 PM

Follow up 1month